

**العمر(تاريخ الميلاد) : 20-05-1981 المهنة : ربه منزل**

التعليق على التشخيص : -----

رئيس  
المجلس الطبي المتخصص

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|                                                                            |                                                                                     |
|----------------------------------------------------------------------------|-------------------------------------------------------------------------------------|
| <b>Name:</b> دعاء حسن احمد عبدالسلام<br><b>National ID:</b> 28105202102206 |  |
|----------------------------------------------------------------------------|-------------------------------------------------------------------------------------|

|           |
|-----------|
| تقرير طبي |
|-----------|

|                  |
|------------------|
| السيد            |
| تحية طبية وبعد,, |

|                       |                       |
|-----------------------|-----------------------|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم:                 | رقم طبي:              |

|                                          |
|------------------------------------------|
| نتشرف بان نوضح فيما يلي تقريرنا عن حالته |
|------------------------------------------|

|                                             |                                             |
|---------------------------------------------|---------------------------------------------|
| تاريخ وساعة الدخول :<br>-- --:-- yyyy-mm-dd | تاريخ وساعة الخروج :<br>PM 01:17 05-07-2026 |
|---------------------------------------------|---------------------------------------------|

|                                        |
|----------------------------------------|
| وبعد الكشف والفحوصات والمتابعة وجد انه |
|----------------------------------------|

|                                                                                                        |
|--------------------------------------------------------------------------------------------------------|
| المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي<br>Second Line Hormonal Treatment Aromatase Inhibitor |
|--------------------------------------------------------------------------------------------------------|

|                         |                                   |
|-------------------------|-----------------------------------|
| التاريخ :<br>05-07-2026 | الوقت :<br>-- --:--               |
|                         | الطبيب المعالج :<br>د/شيماء لاشين |



LD3897591



٢٨١٠٥٢٠٢١٠٢٢٠٦

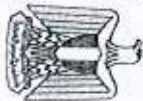
دعاه  
حسن احمد عبدالسلام  
١٩٧ سن تاليف المصنفيه  
مركز كوداسة - الجزيرة

Dms

وفا



٢٠٢٥/٠٥ ٢٨١٠٥٢٠٢١٠٢٢٠٦  
حاصلة على دبلوم تجارة  
انثى  
احمد حسن احمد عبدالحميد  
البيطرة سارية حتى ٢٠٣٤/٠٥/٠٧





جهة الإصدار: فرع الدوحة

تاريخ الإصدار: 2023-3-27

إلى من يهمه الأمر .....

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للانتفاع الخاضعين لقانون التأمين الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "القوى العاملة والمعاشات والإرامل" وكذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنة 2012 "الإطفال دون السن المدرسي" وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعولة .  
وطبقا لقاعدة البيانات المرسلة من التأمينات الاجتماعية  
وطبقا لقاعدة البيانات المرسلة من الضمان الاجتماعي  
والبحث تبين أن

السيد (ة) د.عبد حسن احمد عبد السلام بطاقة رقم 28105202102206  
وطبقا الى خطاب من التأمينات الاجتماعية يفرد عدم الانتفاع  
باعتبار من الفئات غير المنتفعة بالتأمين الصحي صدر هذا الخطاب دون اي مسئولية على الهيئة العامة للتأمين الصحي حال خضوع المواطن لقانون التأمين الصحي الشامل  
وشكرا

الموظف المختص

امتنان نعم

إدارة مركز نظم المعلومات  
بمكتب

تاريخ الطابع: 2023-3-27

مستلم 584730



الخطاب ساري لمدة ثلاثة أشهر من تاريخ الإصدار إلى حالة تغير حالة المواطن من قبل الهيئة القومية للتأمينات الاجتماعية أو وزارة التضامن الاجتماعي أو وزارة التربية والتعليم

الإدارة المركزية لتكنولوجيا المعلومات ودعم القرار



Presidential Program for Women's Health  
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة  
وزارة الصحة والسكان

A-Patient ID:

1- Name : دعاء حسن احمد عبدالسلام

Sex:

3- Medical number:  
Female

83129564  
Old Med :

Old Med :  
804-202

2-ID number: 28105202102206

Patient Age : 43 Y

### MDT form

Family history

Family history  
-ve

Phone

Phone

01154439981

BMI

BMI

REASON 0

Marital status

Marital Status  
MARRIED

NRS Score

NRS Score

GROUP

GROUP  
C

Date of MDT

Co morbidity

Co Morbidity

Menopausal  
status

Menopausal Status  
PREMENOPAUSE

Initial STAGE

Initial STAGE

Breast Imaging :

### Clinical data

Mets Workup :

BX

ER

8/8

PR

6/8

Her 2 Neu

0

SISH

KI67

40

BX COMMENT:

ER COMMENT:

PR COMMENT:

Her 2 Neu COMMENT:

SISH COMMENT:

T

2

N

2

M

0

Sonomammography

RT LOQ mass 8-9 o'clock 2.5\*2 cm

### PATIENT HISTORY

pt received NACT AC x4 ++taxotere x 4  
no remnant of invasive component

rt breast conservative + axillary LN dissection: (14/2, 15).  
remnant of ductal carcinoma insitu  
free margins including extra inferior & deep margins LN +VE 1/13  
partial pathological response YPTis YPN1

### MDT OUTCOME & PLAN

for PORT, zoladex/AI for 5 ys

consider extended , zometa/6 mn for 3 ys after dental

### MEDICAL ONCOLOGY

SURGERY

Psychiatric

MEDICAL ONCOLOGY

د/ هاجر القزويني

SURGERY

Psychiatric

### SIGNATURE

RADIOLOGY

PATHOLOGY

Nutrition

RADIOLOGY

PATHOLOGY

Nutrition



|                 |                                      |                    |                                  |
|-----------------|--------------------------------------|--------------------|----------------------------------|
| Patient Name    | : الأستاذة / دعاء حسن احمد عبدالسلام | Request No.        | : 13007498                       |
| Patient No.     | : 28105202102206                     | Request Date       | : 06/03/2023                     |
| Path No.        | : 707/2023                           | Verification Date  | : 22/03/2023                     |
| Sex             | : Female                             | Financial Category | : اورام دار السلام - نفقة الدولة |
| Age             | : 41 Y                               | Department         | : Out Patient                    |
| Referring Doc : |                                      | Registered by      | : marwa mohamed youss            |

### Pathology Report

#### Clinical data:

Right breast mass with enlarged right axillary lymph nodes

#### Nature of specimen:

Tru cut biopsy, collected at 6/3/2023 , 2:45 pm

#### Gross examination

##### 1)Right breast lesion:

Multiple greyish white tissue cores up to 1.5 cm , totally submitted.

##### 2)right axillary lymph nodes:

Multiple greyish white tissue cores up to 1cm , totally submitted.

Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

#### Microscopic examination:

\* Histological type: Invasive duct carcinoma .

\* Grade ( differentiation +nuclear pleomorphism+mitosis ): (3+2+1) Grade II

\* DCIS: Focal

\*Angioinvasion: Could not be detected within the limits of biopsy recieved

##### 2)Right axillary lymph node:

\* Histological type:Metastatic adenocarcinoma compatible with mammary origin

\*IPT:

ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)

PR: Strong nuclear reaction in 30% of tumor cells (Allred score 6/8)

Her-2: Weak membrane reaction in 10% of tumor cells (score 1+)

Ki 67: 40 % nuclear reaction in tumor cells

Positive control showed appropriate reactivity

Verified by :

dr.esraa magdy





|                 |                                     |                    |                                  |
|-----------------|-------------------------------------|--------------------|----------------------------------|
| Patient Name    | : الأستاذ / دعاء حسن احمد عبدالسلام | Request No.        | : 13007498                       |
| Patient No.     | : 28105202102206                    | Request Date       | : 06/03/2023                     |
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| Sex             | : Female                            | Financial Category | : اورام دار السلام - نفقة الدولة |
| Age             | : 41 Y                              | Department         | : Out Patient                    |
| Referring Doc : |                                     | Registered by      | : .marwa mohamed youss           |

### Pathology Report

**Diagnosis:**

Right breast mass with enlarged right axillary lymph nodes, tru cut biopsy:

1) Right breast:

Invasive duct carcinoma, Grade II.

2) Right axillary lymph nodes:

Metastatic adenocarcinoma compatible with mammary origin

IPT:

ER: Positive (score 8/8).

PR: Positive (score 6/8).

HER-2: (score 1+).

Ki67: Positive in 40% of tumor cells.

Prof. Gamal Abdelatty

Dr.Marwa Mahmoud

Verified by :

dr.esraa magdy

Printing Date : 27/03/2023 08:57:36

Page 2 of 2

Printing By : .marwa mohamed youss



|                 |                                      |                    |                                  |
|-----------------|--------------------------------------|--------------------|----------------------------------|
| Patient Name    | : الأستاذة / دعاء حسن احمد عبدالسلام | Request No.        | : 13008593                       |
| Patient No.     | : 28105202102206                     | Request Date       | : 28/10/2023                     |
| Path No.        | : 3723/23                            | Verification Date  | : 11/11/2023                     |
| Sex             | : Female                             | Financial Category | : اورام دار السلام - نفقة الدولة |
| Age             | : 42 Y                               | Department         | : Out Patient                    |
| Referring Doc : |                                      | Registered by      | : marwa mohamed youss            |

### Pathology Report

Clinical Data: Right breast cancer, post NACT.

Nature Specimen: Right breast conservative surgery and axillary lymph nodes.

#### Gross Examination:

- \* Procedure: Right breast conservative surgery .
- Fibrofatty tissue piece measured 5.5x4.5x2 cm with covering skin ellipse measured 3.5x0.7 cm.
- \* Tumor extent: A wire guided greyish white tumor bed site
- \* Size: 1.9x1.5 cm 0.1 cm from inferior margin .
- \* Another greyish white area measured 2.5x2 cm was located near lateral margin .
- \* Extra inferior margin : 5x4 cm
- \* Extra deep margin : 5x4 cm
- \* Axilla measured 9x7 cm entangling 13 lymph node up to 2x1.5 cm .

#### Microscopic Examination:

- \* No remnants of invasive component .
- \* Ductal carcinoma in situ; 5% remnants .
- \* Resection margins: Free margins ( including the extra inferior and extra deep margin )
- \* Histologic response to therapy: Partial pathological response .
- \* Lymph nodes : Positive in 1/13 .
- \* Frozen section : Inferior and deep margins showed atypical crushed cells .

#### Diagnosis:

Right breast conservative surgery and axillary lymph nodes, post NACT:

- \* No remnants of invasive component .
- \* Remnants of ductal carcinoma in situ .
- Free margins including extra inferior and deep margin .
- Lymph nodes : Positive in 1/13 .

Verified by :

Rana fathy





|                 |                                      |                    |                                   |
|-----------------|--------------------------------------|--------------------|-----------------------------------|
| Patient Name    | : الأستاذة / دعاء حسن احمد عبدالسلام | Request No.        | : 13008593                        |
| Patient No.     | : 28105202102206                     | Request Date       | : 28/10/2023                      |
| Path No.        | : 3723/23                            | Verification Date  | : 11/11/2023                      |
| Sex             | : Female                             | Financial Category | : اورام دالر السلام - نفقة الدولة |
| Age             | : 42 Y                               | Department         | : Out Patient                     |
| Referring Doc : |                                      | Registered by      | : marwa mohamed youss             |

### Pathology Report

- Partial pathological response .
- Stage: ypTis, ypN1
- RCB: 1.64, RCB class II

Signature  
Dr. Rana Fathy  
Dr. Menan Hossam

Verified by :  
Rana fathy



|                                              |                   |
|----------------------------------------------|-------------------|
| Patient Name: Doaa Hassan Ahmed Abd Al Salam | Date : 24/08/2025 |
| Exam : MAMMOGRAPHY BOTH                      | Age : 44 Year(s)  |
| Medical ID: 804/2023                         | Pat. ID: 31907    |

**Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:**

**Clinical summary:** Post right CBS and ALAND followed by radiotherapy, routine follow up study.

**Findings:**

**Right breast.**

- Scattered fibroglandular densities. (ACR b)
- UOQ architectural distortion with no speculations, masses, asymmetry, or suspicious microcalcifications.
- Thickened skin and coarsen trabeculae.
- Ultrasound demonstrated UOQ, area of parenchymal disruption underneath the operative scar with no solid, cystic lesions or fluid collections.
- Thickened skin reaching 7 mm and coarsen trabeculae.
- Normal nipple/areola complex.
- Clear right axilla. No pathological infra or supra-clavicular lymph nodes

**Left breast.**

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses, distortion, asymmetry, or suspicious microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

**Conclusion:-**

**Right breast (BIRADS 2): Post-therapeutic sequelae**

**Left breast (BIRADS 1): Normal study.**

**Recommendation:-**

**For annual follow up.**

Best Regards,  
Dr. Amal Hegazi, M.Sc.

**BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:**

**Category & recommendations**

|                                                 | <u>Suspected Cancer Risk</u> |
|-------------------------------------------------|------------------------------|
| BIRADS 0: Non-informative study.                |                              |
| BIRADS 1: Normal study, routine screening       | 0 %                          |
| BIRADS 2: Benign findings, routine screening    | 0 %                          |
| BIRADS 3: Probably benign, for close follow up. | 2 %                          |
| BIRADS 4: Probably malignant, for biopsy.       | 50 %                         |
| A: mild suspicion.                              |                              |
| B: moderate suspicion.                          |                              |
| C: high suspicion.                              |                              |
| BIRADS 5: Malignant, for biopsy.                | 98-100%                      |
| BIRADS 6: Pathologically proven malignant       |                              |

|                                              |                   |
|----------------------------------------------|-------------------|
| Patient Name: Doaa Hassan Ahmed Abd Al Salam | Date : 31/08/2025 |
| Exam : X-RAY CHEST                           | Age : 44 Year(s)  |
| Medical ID: 804/2023                         | Pat. ID: 31907    |

Plain X-ray **Chest** (PA view) revealed:

- *Clear both lung fields.*
- *Clear both costo-phrenic angles.*
- *Normal cardiac size & configuration.*
- *Normal thoracic cage*

Signature Dr . Nermien Ramadan

31/08/2025 12:54 PM



Name: Doaa Hassan

Date: 10/5/2023

**Contrast enhanced Spectral Mammography**

**CLINICAL SUMMARY:** Right breast cancer pathologically proved to be IDC

**TECHNIQUE:** CC & MLO low energy & recombined (post contrast) images

**FINDINGS:**

Background Uptake: mild background enhancement

ACR: b (scattered fibro-glandular densities)

**RIGHT BREAST:**

- **Low energy (Non contrast) images:** show mid outer irregular hyperdense mass lesion with indistinct margin and intra-lesional punctate microcalcifications.
- **Recombined (Post contrast) images:**
  - ☐ Corresponding heterogeneous moderate mass enhancement is noted, measuring 2.8 x 1.6 x 2.3 cm along its maximum dimensions, being at about 4 cm from the nipple.
  - ☐ Adjacent indeterminate faintly enhancing nodules are noted (on CC view) occupying an area of about 7.8 cm, being at about 1.6 cm from the index lesion.
  - ☐ The previously reported deeply seated central asymmetry shows no notable enhancement (noted in MLO view).
- Normal skin / nipple / areolar complex.
- **Complementary US:** large pathologically enlarged lymph node is noted showing infiltrated fatty hilum. No evidence of pathological supra or infraclavicular lymph nodes.

**LEFT BREAST:**

- **Low energy (Non contrast) images** No masses, asymmetry or distortion or suspicious calcifications.
- **Recombined (Post contrast) images** show, no pathological uptake.
- Normal skin / nipple / areolar complex.

**IMPRESSION:**

**RIGHT BREAST:**

- Mid outer malignant mass associated with adjacent indeterminate faintly enhancing nodules and ipsilateral pathologically enlarged level I axillary lymph node (BIRADS 6= pathologically proven malignancy).
- Right breast deeply seated asymmetry with no notable enhancement (BIRADS 3), for 6 months follow up by single view mammography

**LEFT BREAST:** No pathologically enhancing lesions (BIRADS 1= normal)

*Best regards,*

*Dr. Sherihan Fakhry, MD*



|                                              |                   |
|----------------------------------------------|-------------------|
| Patient Name: Doaa Hassan Ahmed Abd Al Salam | Date : 28/03/2023 |
| Exam : C.T CHEST WITH CONTRAST               | Age : 42 Year(s)  |
| Medical ID:                                  | Pat. ID: 31907    |

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,  
ABDOMEN & PELVIS REVEALED: -**  
**Reported case of right breast cancer**

**1. Chest:**

- Right breast LOQ irregular speculated mass lesion measuring 2.5 X 1.5 cm.
- Right axillary level I and II pathologically enlarged lymph nodes the largest measures 4 X 3 cm.
- no sizable pulmonary nodules.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

**2. Abdomen and pelvis:**

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- IUD seen in place.
- Right adnexal cystic lesion mostly functional.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

**3. The scanned skeleton:**

|                                                    |                                              |
|----------------------------------------------------|----------------------------------------------|
| PATIENT NAME: <b>DOAA HASSAN AHMED ABDEL SALAM</b> | PATIENT ID #: <b>2023221552</b>              |
| GENDER: <b>Female</b>                              | ORDER #: <b>2196186,2196187</b>              |
| AGE: <b>41 y</b>                                   | DATE OF SERVICE: <b>05-Mar-2023 09:39 AM</b> |

**Report: - all studies**

## FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

**INDICATION:**

- Right breast palpable lump.

**TECHNIQUE:**

- CC and MLO view.
- Complementary ultrasound

**REPORT:**

- Scattered fibroglandular densities (ACR B).

**RIGHT BREAST:**

- Right LOQ dense lesion with indistinct margins with overlying loosely attached microcalcifications.
- Other few scattered microcalcific foci.
- The US revealed right 8-9 o'clock irregular hypoechoic lesion with internal vascularity on color doppler measuring about 25 x 18 mm . Its seen touching the under-surface of the skin and located at about 4 cm from the nipple.
- Right LOQ focal deeply seated focal assymetry corresponds to subtle parenchymal distortion seen deeply seated at 9 o'clock.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Right axillary ( level I) pathologically enlarged lymph node measuring about 48.5 x 25.5 mm showing lost hilum most likley amalgamated nodes.
- No infra or supraclavicular pathological lymph nodes.

**LEFT BREAST:**

- No spiculated mass lesions or grouped microcalcifications.
- This was confirmed by Complementary Ultrasound which showed no solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.



**NAME:** Doaa Hassan Ahmed  
**DATE:** 26.10.2023

### **MAMMOGRAPHIC GUIDED STEREOTACTIC WIRE LOCALIZATION:**

#### **CLINICAL SUMMARY:**

Right breast CA that was under NAC, for localization of the LOQ previously inserted clip

#### **TECHNIQUE:**

Under complete sterile conditions, 3ml of local anaesthetic were administered at the sites of concern in the right breast.  
A 21G wire was used to localize the LOQ clip located at 8-9 o'clock, about 5cm away from the nipple, and the wire was inserted 2cm deep with the hook about 2mm from the clip.

Skin marking was also performed.

Post procedure films were done in MLO and CC views to confirm.

**Best regards,  
Dr. Mahi Mokhtar**



|                                              |                   |
|----------------------------------------------|-------------------|
| Patient Name: Doaa Hassan Ahmed Abd Al Salam | Date : 04/09/2024 |
| Exam : MAMMOGRAPHY BOTH                      | Age : 43 Year(s)  |
| Medical ID: 804/2023                         | Pat. ID: 31907    |

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION  
ULTRASOUND**

**Indication:**

Patient had CBS in right breast.

**Technique**

- Bilateral MLO and CC views
- Bilateral breast ultrasound

**Report:**

- Scattered fibro glandular density (ACR B).
- No left breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Right breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

**Opinion:**

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Left breast benign finding (BIRADS 2)

Signature Dr. Nehal Hamdy

04/09/2024 5:19 PM